Advanced MET exon 14 skipping lung cancer NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Swelling and fluid retention

☐ Lung inflammation, which can be serious or fatal

☐ Creatinine and other blood-test changes

☐ Nausea, diarrhoea and vomiting

☐ Liver test changes and possible liver injury

☐ Heart rhythm changes

Discuss the advanced-disease treatment aim, possible benefit and uncertainty, alternatives including not proceeding, daily tablets with food, swelling and bowel effects, serious lung/liver/heart risks, monitoring, pregnancy and fertility, and the right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

Confirm the MET result, previous treatment, dose, other medicines and emergency contact. Check blood count, electrolytes, kidney and liver function; ECG if clinically indicated. Discuss pregnancy, contraception and breastfeeding, and perform a pregnancy test when applicable. Repeat blood count, electrolytes, kidney and liver tests at the team’s agreed reviews. ECG is used when clinically indicated. Report new lung symptoms, swelling, diarrhoea or vomiting between visits. A rise in creatinine needs assessment because the medicine can change the test result without necessarily damaging the kidneys. Scans follow the Consultant’s plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending